



Pregnancy if You Have Diabetes

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If you have **diabetes** and plan to have a baby, you should try to get your blood glucose levels, also called blood sugar levels, close to your target range before getting pregnant. High blood glucose levels can harm your baby during the first weeks of pregnancy. Talk with your primary health care professional or [obstetrician](#) about how to prepare for pregnancy when you have diabetes. Your primary health care professional may be a doctor, physician assistant, or nurse practitioner. If you have diabetes and find out you are pregnant, see your health care professional as soon as possible.

How should I prepare for pregnancy if I have diabetes?

Pregnancy is something that you should try to plan if you have diabetes. You should talk with your health care professional about [preconception care](#) . Your health care professional can help you plan for pregnancy and lower the risk of health problems for you and your baby.

Before you get pregnant, have a complete medical checkup. If you are planning to get pregnant your blood glucose targets may be different. Your health care professional may recommend that you see a [registered dietitian](#). A dietitian can teach you what, how much, and when to [eat or drink](#) to manage your blood glucose level and reach or stay at a [healthy weight](#). Your insurance can tell you if this service is covered.



Plan to manage your blood glucose levels before you get pregnant.

Your health care professional may also prescribe different medicines that are safe during pregnancy.

Before getting pregnant, aim to make [physical activity](#) a regular part of your life. Try for at least 150 minutes of [moderate-intensity physical activity](#), such as brisk walking, each week.¹

If you and your partner are finding it hard to get pregnant, talk with your health care professional to find out if you need any further evaluation. Diabetes may produce health problems such as [erectile dysfunction](#) and [infertility](#) [↗](#).

What should I do if I have diabetes and I am pregnant?

If you have diabetes and you find out that you are pregnant, talk with your health care professional as soon as possible about how to manage your diabetes during your pregnancy. Your target blood glucose level range may be different when you are pregnant than before pregnancy. Working with your health care team and following your plan to manage diabetes can help you have a healthy pregnancy and a healthy baby.

Some people may have diabetes and not know it. If you have [risk factors for type 2 diabetes](#), your health care professional may [test you for diabetes](#) in the first prenatal visit. Some people develop diabetes during pregnancy that goes away after they have the baby. This type of diabetes is called [gestational diabetes](#). Health care professionals usually diagnose gestational diabetes between weeks 24 and 28 of pregnancy.^{2,3} However, having gestational diabetes increases your risk of getting type 2 diabetes later in life.

Find out if you had diabetes before getting pregnant, or if you have gestational diabetes. These two types of diabetes are managed differently.

How can my diabetes affect my baby?

A baby's organs, such as the brain, heart, kidneys, and lungs, start forming during the first 8 weeks of pregnancy.³ High blood glucose levels can harm the baby during this early stage and increase the chance that your baby will have birth defects of any of these organs.

High blood glucose levels during pregnancy can also increase the chance that your baby will

- be born too early
- be above average in size
- have breathing problems right after birth
- have low blood glucose levels, also called [hypoglycemia](#), right after birth

A health care professional may recommend keeping the baby in a special care unit after delivery if there is a chance that the baby may develop low blood glucose levels or other health problems. Babies that are born too early, are very small, or have other serious health problems may also need special care.

High blood glucose levels can also increase the chance that you will have a [miscarriage](#) [NIH](#) or a stillborn baby. Stillborn means the baby dies in the womb during the second half of pregnancy.

If you have diabetes and your blood glucose levels are high, your baby also has an increased risk of developing some health problems later in life, such as [obesity](#) or [type 2 diabetes](#).

How can my diabetes affect me during pregnancy?

Changes to your [hormones](#) and your body during pregnancy affect your blood glucose levels. Even if you've had diabetes for years, you may need to adjust your meal plan, physical activity routine, or medicines. If you were not taking [insulin](#) before your pregnancy, you may need to add insulin. It is common during pregnancy to need to increase your insulin dose as you get closer to your due date. Your blood glucose levels may change quickly during your delivery and after you have your baby. Your health care team may be checking your blood glucose levels often to adjust your treatment.

What health problems could I develop during pregnancy because of my diabetes?

Pregnancy can make certain **diabetes health problems** worse, such as eye, heart, and kidney disease, especially if your blood glucose levels are too high.

You also have a greater chance of developing [preeclampsia](#) [NIH](#) . In preeclampsia, your blood pressure goes up and you may have too much [protein](#) in your [urine](#) during the second half of pregnancy.

People with diabetes may have babies who are above average in size. If your baby is above average in size, your health care team may recommend [cesarean delivery](#) [NIH](#) .

How should I manage my diabetes during pregnancy?

To help you and your baby stay healthy

- follow your diabetes care plan
- get checkups
- take your vitamins
- stop smoking

Work with your health care team

You are the most important member of your health care team because you are the one who manages your diabetes every day. You should have regular visits with members of your health care team who are experts in diabetes and pregnancy. Asking for advice and letting your health care team know if you have any concerns will help you and your baby get the best care. Your health care team may include

- a health care professional who is an expert in diabetes care, such as an [endocrinologist](#) or a [diabetologist](#)

- an obstetrician with experience treating people with diabetes
- a [diabetes educator](#), nurse, or dietitian who can tell you more about how to manage your diabetes, including what to eat and drink
- specialists who diagnose and treat diabetes health problems, such as eye, kidney, and heart diseases
- nurses, social workers, and psychologists to help you cope with stress, worry, and the extra demands of pregnancy



You should have regular visits with members of your health care team who are experts in diabetes and pregnancy.

Get checkups

Have a complete checkup as soon as you know you are pregnant.

Pregnancy can make some diabetes health problems worse. To help prevent developing or worsening health problems related to your diabetes, your health care team may recommend adjusting your treatment.

Your health care team will follow your blood pressure closely because diabetes puts you at risk of developing preeclampsia. If you have or are concerned about developing [high blood pressure](#) [NIH](#), ask your health care professional what your blood pressure goal should be during pregnancy. Your health care professional may recommend changes to your blood pressure medicines before pregnancy or early in pregnancy.

[Diabetic eye disease](#) can get worse during your pregnancy, and you may need special care. During your pregnancy, you may have frequent [dilated eye exams](#) [NIH](#) to check your eye health. After you have your baby, your eye doctor may follow up closely for a year.

Your health care team may check your kidneys at the beginning of your pregnancy. If you already have [kidney disease](#), your team may follow up more closely.

Let your health care team know if you have any changes in the sensation of your legs or arms. Diabetes can [affect your nerves](#), especially in your legs and arms. Pregnancy can worsen nerve damage because the baby can press on nerves already damaged by diabetes.

You may also get other routine pregnancy care, such as appropriate [vaccines](#) and screenings for [genetic](#), [thyroid](#), or [infectious diseases](#) [NIH](#) that can affect you or your baby.

Don't smoke

[Smoking](#) can increase your chance of having a stillborn baby or a baby born too early. Both smoking and diabetes can make your blood vessels narrow. [E-cigarettes](#) [NIH](#) aren't a safe option either. If you smoke, vape, or use other tobacco products, it's best to stop as soon as possible. [Ask for help](#) so you don't have to do it alone.

See a dietitian

If you don't already see a dietitian, you should start seeing one. Your insurance can tell you if this service is covered.

During pregnancy, a dietitian may help you make changes to your meal plan, such as adding extra [calories](#), [protein foods](#) , and other nutrients. You may need to see your dietitian more often during pregnancy as your needs change.

Trying to lose weight during pregnancy may not be healthy. However, you should ask your health care professional what amount of weight gain they recommend for you.

Be physically active

Physical activity may help lower your blood glucose, blood pressure, and [cholesterol](#) [NIH](#)  levels. Being active may also help you get better sleep and improve your mood.

Talk with your health care team about what activities are safe for you [during your pregnancy](#). The *[Physical Activity Guidelines for Americans, 2nd Edition](#)*  (PDF, 15.6 MB)  recommends that during and after pregnancy, most people should try to get at least 150 minutes of [moderate-intensity physical activity](#)  a week and include strength-training activities twice a week.¹ You may need to adjust your level of activity based on how active you were before your pregnancy.



Talk with your health care team about what activities are safe for you during pregnancy.

Avoid alcoholic beverages

You should [avoid drinking alcoholic beverages](#) [NIH](#) throughout pregnancy. Drinking alcoholic beverages can lead to health problems for you and your baby.

Adjust your medicines

Some [medicines](#) are not safe during pregnancy. Tell your health care professional about all the medicines you take, including prescription and over-the-counter medicines and supplements. Your health care professional can tell you which medicines to stop taking or may prescribe different medicines that are safe during pregnancy.

To manage diabetes during pregnancy, health care professionals often prescribe insulin, along with [medical nutrition therapy](#). Medical nutrition therapy includes counseling to help you create and follow a meal plan. Ask your health care team or your insurance company if your benefits include medical nutrition therapy.

If you're already taking insulin, you might need to change the type, the amount, or how and when you take it. You may need less insulin during your first trimester, but probably will need more as you go through pregnancy. Your insulin needs may double or even triple as you get closer to your due date and will drop quickly after you have your baby. Your health care team will work with you to create an insulin routine to meet your changing needs. Health care professionals may prescribe oral medicines to manage diabetes in some cases.

Diabetes medicines, technologies, and office visits can be expensive. Tell your health care professional or health care team if you are [having trouble affording diabetes care](#) or pregnancy care. A health care professional, diabetes educator, or social worker may be able to tell you about resources that provide financial help for diabetes care and pregnancy care.

Ask your health care team if you should take multivitamins, [folic acid](#), [iron](#), or [calcium](#) [NIH](#) supplements. Your health care team may suggest a low dose of aspirin to lower your risk of preeclampsia. Talk with your health care team to learn more about low-dose aspirin.

What do I need to know about blood glucose testing during pregnancy?

Your blood glucose level target ranges, when, and how often you check your blood glucose levels may change during pregnancy. Checking your blood glucose level may help you manage how high or low they get. Ask your health care team how often and at what times you should check your blood glucose levels.

You can check your blood glucose level at home with a [blood glucose meter](#) . The meter tests a drop of your blood, usually from a prick of your fingertip. The meter will show you how much glucose is in your blood at that moment. If you take insulin to manage your diabetes, you may benefit from using a [continuous glucose monitor \(CGM\)](#).

Your health care team may ask you to check your urine at home for substances called ketones. High levels of [ketones](#) in your urine may mean that you need to pay more attention to how you are managing your diabetes. Work with your health care professional to change your diabetes care plan. If high levels of ketones are left untreated, you may develop a dangerous condition called [diabetic ketoacidosis](#) .



During your pregnancy, you may need to check your blood glucose levels more often.

Target blood glucose levels during pregnancy

Your daily blood glucose targets may be different during pregnancy than before pregnancy. Ask your health care team which targets are right for you.

You can keep track of your blood glucose levels using [My Daily Blood Glucose Record](#) (PDF, 44 KB) . You can also use an electronic blood glucose tracking system on your computer or mobile device. Record the results every time you check your blood glucose levels. Your blood glucose records can help you and your health care team make changes to your meal plan, physical activity level, or diabetes medicines. You also can make notes about your insulin needs and ketones. Take your tracker with you when you visit your health care team.

Health care professionals most often recommend the following blood glucose level targets during pregnancy^{3,4}

- fasting, before meals, at bedtime, and overnight: 70 to 95 mg/dL
- 1 hour after eating: 110 to 140 mg/dL
- 2 hours after eating: 100 to 120 mg/dL

Ask your health care team what targets are best for you.

If you are using a CGM, the most often recommended target range for pregnancy is 63 to 140 mg/dL. You will need to change this target range in your CGM settings. You can ask your health care team what percentage of time you should spend in this range and above and below this range.

A1C blood test

Another way to check your blood glucose levels is to have an [A1C blood test](#). Results of the A1C test reflect your average blood glucose levels during the last 3 months. Changes in your body during pregnancy affect A1C levels. A1C test is not the best test to monitor your blood glucose level during pregnancy. Most people with diabetes should aim for an A1C of 6.5% or lower just before pregnancy. After the first 3 months of pregnancy, your target may be as low as 6%.^{3,4} These targets may differ from the A1C goals you've had in the past. Your health care team can help you set the A1C targets that are best for you.

What tests will check my baby's health during pregnancy?

You should have prenatal tests throughout your pregnancy. Your health care professional will do [ultrasounds](#) [NIH](#) and [nonstress tests](#) [NIH](#) to check your baby's health. Talk with your health care team about other prenatal tests you'll have and when you might have them.

How can my diabetes affect me after I have my baby?

You also need special care after you have your baby.

- Blood glucose levels can drop very quickly after delivery. You should ask your health care team what changes should be made to your medicines. You should also check your blood glucose levels often to watch for low blood glucose levels in the first few days.
- If you had gestational diabetes, your health care team will check your blood glucose levels in the first 3 months after having your baby to see if your gestational diabetes has completely gone away.⁴ If you have gestational diabetes, you are more likely to develop type 2 diabetes later in life.
- You should also check your blood pressure or have it checked by a health care professional during the first 3 to 12 weeks after having your baby.^{5,6}
- Diabetic eye disease may get worse after you have a baby. Ask your eye doctor how often you need to have eye exams.

[Breastfeeding](#) [NIH](#) has benefits for your baby, such as improving nutrition and defenses against some diseases. It also may benefit you by helping you to lose weight after delivery. However, breastfeeding increases your risk of low blood glucose levels at night. If you are planning to breastfeed your baby, talk with your health care team about how to prevent low blood glucose levels.

Ask your health care team about options for family planning and how soon you can begin to use the [method of your choice](#) [NIH](#).

Clinical Trials on Pregnancy and Diabetes

NIDDK conducts and supports clinical trials in many diseases and conditions, including diabetes. The trials look to find new ways to prevent, detect, or treat disease and improve quality of life.

[Find out if clinical studies are right for you](#) [NIH](#).

What are clinical trials for pregnancy and diabetes?

Clinical trials—and other types of [clinical studies](#) [NIH](#)—are part of medical research and involve people like you. When you volunteer to take part in a clinical study, you help doctors and researchers learn more about disease and improve health care for people in the future.

Watch a video of NIDDK Director Dr. Griffin P. Rodgers explaining the importance of participating in clinical trials.



Why Should I Join a Clinical Trial?

National Institute of Diabetes and Digestive and Kidney Diseases (NIDDK)

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Watch on

What clinical studies for pregnancy and diabetes are looking for participants?

You can view a filtered list of clinical studies on pregnancy and diabetes that are federally funded, open, and recruiting at [ClinicalTrials.gov](https://clinicaltrials.gov) [NIH](#) [↗](#). You can expand or narrow the list to include clinical studies from industry, universities, and individuals; however, the National Institutes of Health does not review these studies and cannot ensure they are safe for you. Always talk with your primary health care professional before you participate in a clinical study.

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